جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : اليصابات ابراهيم موسي ابراهيم
المحافظة : القاهره **الحي / المدينة :** المطريه **العنوان :** 9ش احمد شعراوي
رقم البطاقة : 25002190100721 **رقم التليفون :** 01225534990 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 19-02-1950 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة [11129] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: اليصابات ابراهيم موسي ابراهيم
National ID: 25002190100721

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
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جوستاف روسي

٢٩٢٤

Patient's Name: AL YASABAT IBRAHIM MOUSA
Date: 2026-04-04
Referred by: Prof.Dr. CAP+C
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,

ABDOMEN AND PELVIS REVEALED:

Known case of breast CA. Compared to the previous study dated 7-10-2025, the current study showed:

I- CHEST:

- No appreciable time interval changes regarding the previously seen left breast skin thickening as well as the left breast calcifications.
- Still noted left lingular reticulations likely post radiation sequelae.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- No appreciable time interval changes regarding the previously seen two large calcified uterine lesions measuring 73 x 76 mm and 72 x 45 mm likely fibroids.
- Normal appearance of the rest of organs.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Still noted spondylodegenerative changes of the spine with minimal anterolisthesis of L3 over L4 vertebra and anterior wedging of L1 vertebral body.

Opinion:

❖ **Rather stationary course since last study.**

Dr.sherif Wagdy

Dr.sherif Wagdy

CT Scan
[[DOCTYPE:CT_SCAN]]

Name: اليصابات ابراهيم موسي	
MR. : 2323	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 76-year-old individual with a history of metastatic breast cancer (MBC) to bone, estrogen-receptor positive (ER 8/8), progesterone-receptor positive (PR 8/8), HER2-negative, and Ki-67 > 40 %. The disease course began in 2015 with a left breast-conserving surgery, followed by chemotherapy and placement of a port. An aromatase inhibitor (AI) was continued until March 2023, after which bone metastases were documented. The patient was treated with letrozole (FEMARA) and palbociclib (IBRANCE) until December 2023, then received denosumab (DP). From January 2024, therapy was switched to fulvestrant (FASOLDEX) and subsequently to exemestane (AROMASIN) due to drug unavailability, continued until July 2025. A newly developed osseous lesion prompted initiation of everolimus (AFINITOR) combined with exemestane (AROMASIN) starting September 2025, with plans to add zoledronic acid (ZOMETA) after laboratory results.

The treatment plan was adjusted to include everolimus 5 mg and exemestane 25 mg together with zoledronic acid after confirming calcium levels.

On 4 April 2026, the multidisciplinary team reviewed the case and recommended continuation of exemestane and everolimus without zoledronic acid; the patient had previously received monthly zoledronic acid from September 2025 for seven months. The order for exemestane and everolimus was confirmed for that day.

A bone scan performed on 27 April 2026 showed only equivocal tracer uptake in the right pelvic and acetabular bone, and continuation of everolimus and exemestane was advised.

On 14 June 2026, the regimen was specified as everolimus 10 mg, exemestane 25 mg, and zoledronic acid pending laboratory results, to be dispensed by the daycare pharmacy.

On 15 August 2026, the patient presented for follow-up. Laboratory values were creatinine 1.48 mg/dL, AST 20 U/L, ALT 13 U/L. Physical examination revealed erythema of the breast due to itching and right upper-leg edema without cellulitis. The patient was instructed to continue physiotherapy for three more months and to maintain therapy with exemestane and everolimus after obtaining CBC and total bilirubin results.

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : علياء احمد أبو النصر شهاب الدين

يعتمد ,

Ref.Physician Prof : Goustay Roussey Egypt

Tc-99m MDP SKELETAL SCINTIGRAPHY

History

- 76-year-old female patient with history of left breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole-body scans were acquired 2 – 3 hours post injection of 20 mCi of the tracer.

Findings

Limited comparison with the prior study (images were not provided):

- The previously documented osseous uptake at the right acetabulum is still noted, for clinical/radiological correlation.
- Extensive degenerative changes are noted at different levels of the spine.
- No other osseous lesions could be detected.

IMPRESSION

The prior study report was revised:

- No conclusive scintigraphic evidence of skeletal metastases.
- Equivocal tracer uptake pattern at the right pelvic bone/right acetabulum, for radiological correlation.
- Follow-up imaging only if clinically indicated.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

NILE DIAGNOSTIC CENTER

Call Center 19



Bone Scan
[[DOCTYPE:BONE_SCAN]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : فيصليات ابراهيم موسى ابراهيم Sex: Female
3- Medical number: 83124103
Old Med : 407/2023

2-ID number: 35002190100721 Patient Age : 74 Y

Family history	Family History	Phone	Phone 01286841285	BMI	BMI	REASON OF MDT	REASON OF MDT
marital status	Marital Status	NRS Score	NRS Score	GROUP	GROUP	Date of MDT	Date Of MDT 09/01/20
Co morbidity	Co-morbidity	Menopausal status	Menopausal Status	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :

Mets Workup :
MBC TO BONE
HX OF BREAST SINCE 2015

BX	ER	PR	Her 2 Neu	SISH	KI67
IDC	8/8	8/8	-ve		>40
BX COMMENTS	ER COMMENTS	PR COMMENTS	Her 2 Neu COMMENTS	SISH COMMENTS	

T	N	M
		1 BONE

Sonomammography

PATIENT HISTORY
RECEIVED 6 CYCLES TC THEN KEPT UNDER AROMASINE
RECEIVED RTH
NEW BONE SCAN-->PROGRESSIVE

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN
for faslodex single agent ,,, zometa every month

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

MDT Form
[[DOCTYPE:MDT_FORM]]

وحدة دلالات الأورام
قسم الباثولوجيا - القصر العيني
ت: ٢٣٦٨٢٩٨٤ (٠٢)

IMMUNOHISTOPATHOLOGY

Patient Name: البصيلات ابراهيم

lab.No:2015-246

Sex: Female

Markers: ER, PR, Her 2 neu and Ki67 .

Procedure
Sections from paraffin blocks were treated with monoclonal antibodies against ER, PR, Her 2 neu and Ki67 using streptavidin biotin peroxidase system. DAB was used as substrate as well as chromogen. Hematoxylin was used as counter stain

Results:
Examination of sections treated for ER and PR revealed strongly positive nuclear staining of tumor cells by chromogen.

Examination of sections treated for Her 2 neu revealed no membranous staining of tumor cells by chromogen

Examination of sections treated for Ki67 revealed positive nuclear staining in >40 % of tumor cells by chromogen.

DIAGNOSIS:

- POSITIVE FOR ER AND PR SCORE 8/8
- NEGATIVE FOR HER 2 NEU
- HIGH PROLIFERATIOIN INDEX

PROF. DR. SAMIA GABAL

For T C X Cycles

Immunohistochemistry
[[DOCTYPE:IHC]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Al Yasabat Ibrahim Mousa Ibrahim	Date : 02/02/2025
Exam : C.T CHEST	Age : 75 Year(s)
Medical ID: 407/2023	Pat. ID: 30038

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Compared to previous study dated 03/09/2024

Reported case of breast cancer:

1. *Chest:*
   - Stationary course of left breast scattered coarsely calcified lesions, largest measuring 2.1 x 1.5 cm likely dystrophic calcification with no sizable recurrent lesions or pathological axillary lymph nodes.
   - **Still noted** left upper lung lobe and lingula reticulations, likely post radiation sequel.
   - **Stationary course** of left lower lung lobe anterior segment nodule measuring 5 mm.
   - No pathological hilar or mediastinal lymph nodes.
   - No evidence of pleural effusion, pleural thickening or masses seen.
   - Average cardiac size. No pericardial effusion detected.
   - Normal CT appearance of the thoracic aorta and the great vessels.
   - Bilateral elastofibroma dorsi.
2. *Abdomen and pelvis:*
   - The liver is average in size with homogenous CT texture. No definite focal lesions (on non-contrast basis) or contour irregularities.
   - No intra-hepatic or extra-hepatic biliary radicles dilatation.
   - Normal size & parenchymal texture of the spleen with no focal lesions seen.
   - No pathological abdominal or pelvic lymph nodes.
   - Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
   - Normal appearance of the pancreas and both supra-renals.

CT Scan
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Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Al Yasabat Ibrahim Mousa Ibrahim	Date : 02/02/2025
Exam : C.T CHEST	Age : 75 Year(s)
Medical ID: 407/2023	Pat. ID: 30038

- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- **Stationary course** of multiple large coarsely calcified uterine focal lesions mostly fibroids the largest measures 9.7 X 8 cm.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. *The scanned skeleton:*
   - **Stationary course of** right ischial / inferior pubic ramus sclerotic lesion.
   - **Stationary course** of Diffuse osteopenia with L1 anterior wedging and mild retropulsion.
   - No extra-osseous soft tissue component.

OPINION:

Rather stationary course since the last study.

Dr/Ali Saber, MSc.

Dr. Mohamed Mohsen, M.D.

CT Scan
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National ID – Front & Back







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